MAF Blood Chemistry Key Association Category XX – Menopausal Women

- Check for systemic inflammation
- Check hormone levels

MAF Nutritional Key Category XX – Menopausal Women*

*Nutritional Support for Progesterone Function:**

1. Progestaid™◊ (K4): 1-2 capsules with meals.
2. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

   Alternative Formulas
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§

§Currently unavailable

*Nutritional Support for Estrogen Metabolism:**

1. Estrovite™◊ (K5): 1-2 capsules with meals.
2. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

   Alternative Formulas
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§
3. DHEA-25C™◊ (K141): 1 capsule once a day.

   Alternative Formula
   - DHEA (K18): 10 drops (0.255 mL) 1-2 times a day.

§Currently unavailable

*Nutritional Support for Progesterone Function:**

1. Pregnenolone (K19): 10 drops (0.255 mL) 1-3 times a day.

Progestaid™◊ (K4) is a natural herbal and nutritional product to support luteal function and progesterone output.* Estrovite™◊ (K5) is a multi-use vitamin, mineral, and herbal product to support estrogen receptor activity, support a detoxification of estrogens, and support estrogen metabolism.* Super EFA Complex™◊ (K8) offers a variety of essential fatty acids to support female hormone metabolism.*

Part III: Foods

This category screens for foods that have both a positive and negative impact on metabolism, such as essential fatty acids, lignans, dietary fiber, caffeine, and partially hydrogenated fats. The individual's consumption of raw nuts, seeds, and fish will give the clinician a general idea of the amount of essential fatty acids (EFAs) in his/her diet. The clinician needs to clarify that toasted or flavored nuts, canned tuna, and fried fish do not count as a source of EFAs. If the individual has a low dietary intake of EFAs, an EFA supplement should be recommended. Most individuals will require an omega-3 supplement versus an omega-6 source.

The individual's consumption of vegetables needs to be identified. Vegetables are necessary to provide a source of fiber and lignans that are crucial in managing metabolism. These substances have a positive impact on the gastrointestinal tract and its influence on hormone metabolism. This is especially true in elevated hormone patterns. The source of dietary fibers from plants has the potential to inhibit the activity of gastrointestinal beta-glucuronidase, which deconjugates estrogens conjugated in the liver.

The enzyme beta-glucuronidase is found in the large intestine and has the potential to allow hormones to be reabsorbed and remetabolized if the diet is low in fiber. Cruciferous vegetables also contain a chemical component called indole-3-carbinol, which can promote C-2 hydroxylation over C-4 and/or C-16 hydroxylation of estrogens. This will shift estrogen metabolism ratios to 2-hydroxyestrone, which has low estrogenic activity compared to 4-hydroxyestrone or 16 alpha-hydroxyestrone, which has powerful estrogenic activity. Fiber and lignans in the diet also increase the circulating concentrations of sex hormone-binding globulin (SHBG), thus reducing levels of unbound, active hormones.

The consumption of caffeine can have an adverse impact on metabolism at multiple levels. First, caffeine is an adrenal stimulant and can promote overactivity and shift pregnenolone away from androgen synthesis into glucocorticoid synthesis. Second, caffeine increases the demand placed on hepatic detoxification and can deplete the substrates required for hormone detoxification by the cytochrome P450 family. Third, caffeine can have an adverse impact on gastrointestinal health. If an individual suffers from metabolic alterations and is not willing to stop his/her intake of caffeine, he/she will respond very slowly. Please note that decaffeinated coffee still contains 60% caffeine, and is not an acceptable alternative. It is important to educate individuals that their dependence on caffeine may be resolved if they support their adrenal glands and stabilize their glucose levels during the day.

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For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

Fast food consumption on a regular basis can make handling metabolic issues very difficult. Fast food can have an adverse impact on all the major systems needed to promote optimal hormone function. Most fast foods are high in trans-fatty acids and will have a negative impact on EFA metabolism and gastrointestinal health. Educating the individual to choose foods such as salads (without dressings that contain partially hydrogenated oils), quality fish, and unprocessed natural foods can have a profound impact on correcting altered metabolic patterns.

The individual's classification of the three healthiest and three worst foods eaten during an average week is a quick and easy way to understand how he/she views the quality of foods in his/her diet. Many times, individuals are uninformed about healthy foods.

The identification of the individual's stress level is also important to understand. Management of stress can be a powerful tool in establishing good health and function. Ask the patient:

- How many alcoholic beverages do you consume per week?
- How many caffeinated beverages do you consume per day?
- How many times do you eat out per week?
- How many times do you eat raw nuts or seeds per week?
- How many times do you eat fish per week?
- How many times do you work out per week?
- List the three worst foods you eat during the average week.
- List the three healthiest foods you eat during the average week.
- Rate your stress level on a scale of 1-10 during the average week.

Part IV: Medications

This category of the questionnaire allows the clinician to screen the individual's medication intake. It gives the clinician clues to which systems are imbalanced. In addition, it allows the clinician to consider any drug-nutrient interactions.

- List any medications currently taking and for what conditions
- List any supplements currently taking and reason

Antacids

Individuals taking antacids need to be screened for hypochlorhydria; *H. pylori* inoculation; parasitic inoculation; gastrointestinal IgA-mediated food sensitivities; hypothalamus and/or pituitary hypofunction, which alters the release of gastrin; and reactive hypoglycemia, which causes the depletion of essential cofactors for HCl production. It should be noted that when people take antacids and alkalinize their gastrointestinal tract, the absorption of essential nutrients such as zinc, calcium, vitamin B_{12}, and iron may be compromised. In addition to alterations in nutrient absorption, the alkalinizing effect of antacids also may alter proper digestion by compromising the release of cholecystokinin. The end result of antacid consumption may be improper digestion of food due to an inhibitory release of hydrochloric acid, pancreatic enzymes, and bile.

Antibiotics

Individuals taking antibiotics need to be screened for healthy gastrointestinal immunity and dysbiosis. Supporting the body with beneficial bacteria should always be considered.

Antifungals

Individuals taking antifungals need to be screened for alterations in healthy immunity. Antifungals inhibit the steroid hormone enzyme 17,20-lyase, which will shut down the synthesis of DHEA, estrogen, and testosterone.

Antihistamines

Individuals that require antihistamines need to be screened for food sensitivities, hypochlorhydria, adrenal function, and EFA metabolism function.

Antidepressants

Individuals taking antidepressants need to be screened for thyroid imbalances, adrenal insufficiency, blood sugar disorders, and the need for serotonin cofactors, such as 5-HT, ascorbic acid, vitamin B_5, folic acid, iron, and niacin.

*** This statement has not been evaluated by the Food and Drug Administration. This product is not intended to diagnose, treat, cure, or prevent any disease.**

Apex Energetics™ products are intended for nutritional use and health maintenance only and should not replace or delay the use of any medication.

Insulin Management Agents

Individuals taking medications to support proper clearance of glucose or diabetics who require insulin need to be screened for adrenal cortical hyperfunction. The management of physiological inflammation and the proper intake of EFA in the diet need to be considered.

NSAIDs and Anti-Inflammatory Agents

Individuals taking anti-inflammatory agents need to be screened for proper adrenal function, EFA metabolic defects, zinc need, magnesium need, or a need for vitamin B_6.

Anxiety Medications

Individuals taking anxiety medication need to be screened for proper hypothalamus-pituitary-adrenal function, B vitamin status, gastric reflux, hypochlorhydria, and proper breakdown of catecholamines by methylation.

Blood Pressure-Lowering Agents

Individuals taking high blood pressure medications should be screened for adrenal gland hyperfunction, gastrointestinal dysfunction, food sensitivities, EFA deficiencies, magnesium need, etc. High blood pressure medications are categorized into diuretics, ACE inhibitors, beta blockers, and calcium channel blockers.

Cholesterol-Lowering Agents

Individuals with high cholesterol must be ruled out for hypothyroidism, dysinsulinism, gastrointestinal imbalances, and biliary stasis. Cholesterol-lowering drugs can be divided into three main categories: HMG CoA reductase inhibitors, bile acid sequestering resins, and fibric acid derivatives.

Oral Contraceptives

Individuals taking oral contraceptives need to be evaluated for thyroid underconversion and estrogen dominance.

Hormones

Individuals taking hormones should have liver and biliary support to promote a healthy hormone balance.

Laxatives

Individuals taking laxatives need to drink adequate water, exercise, and have adequate fiber in their diet. In addition, the clinician must rule out thyroid hypofunction, hypochlorhydria, biliary stasis, dysbiosis, and food sensitivities.

Diuretics

Individuals taking diuretics are given the medication to remove water from the body. They are used to lower blood pressure in people with hypertension and to assist the heart to pump more efficiently in people with congestive heart failure. Diuretics are also used to reduce water accumulation caused by other diseases. Diuretics will induce thiamine and magnesium deficiency. All individuals taking diuretics should be considered for a source of thiamine and a source of magnesium. A broad-spectrum cardiovascular supplement should also be considered.

Cortisone

Individuals taking cortisone for inflammatory conditions need to have EFA imbalances and adrenal insufficiency ruled out. Individuals taking corticosteroids for autoimmune conditions need to be screened for allergies, sensitivities, and gastrointestinal imbalances. Cortisone will inhibit the body's HPA axis and suppress the body's natural ability to produce glucocorticoids. Cortisone will suppress humoral immunity and IgA cell production and will induce deficiencies of magnesium, melatonin, potassium, vitamin B_6, vitamin C, vitamin D, vitamin K, vitamin A, and zinc. Individuals taking cortisone-containing agents, such as pills, injections, or creams, will induce false elevations of cortisol if an adrenal salivary index is conducted.

The lab tests and blood markers referenced on this form are solely intended for nutritional evaluation within the reference range. Blood test markers that are out of range may require pharmaceutical intervention or other medical treatment and will require that the individual be seen by a medical practitioner with a scope of practice to diagnose, treat, and cure disease.

Symptom groups listed here and on the MAF are not intended to be used as a diagnosis of any disease or condition. For medical evaluation of symptoms, especially frequent or severe symptoms, please seek appropriate conventional medical management by a physician. For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

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